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/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 22 of 117



A 36-year-old woman with hypertension and peripartum cardiomyopathy received a durable LVAD 3 months ago. She has not been re-hospitalized, but she reports feeling fatigued. Medications: warfarin 3 mg daily, aspirin 81 mg, valsartan 40 mg daily, carvedilol 3.125 mg bid, furosemide 40 mg daily. On exam, doppler mean arterial pressure 108 mm Hg, jugular venous pressure 10 cm, clear lungs, VAD sounds with occasional S1S2, no hepatomegaly, warm extremities with trace edema. Device interrogation reveals occasional low flow alarms, but no pump stoppages.

Which of the following is the best next step in management?

A. Start empagliflozin

 B. Start spironolactone

 C. Increase valsartan

D. Stop aspirin

E. Increase carvedilol

Commentary

Testing Point: The Board-certified AHFTC specialist should recognize and treat inappropriately high afterload in LVAD patients.

Correct Answer: C

Rationale:

Patients with durable continuous-flow LVADs should maintain a Doppler mean arterial pressure below approximately 80–90 mmHg, as elevated afterload reduces pump flow and increases the risk of low-flow alarms, pump thrombosis, stroke, and worsening heart failure symptoms. This patient has a Doppler opening blood pressure of 108 mmHg, making uncontrolled hypertension the most likely cause

of her fatigue and low-flow alarms. Increasing Valsartan provides targeted afterload reduction and is the most appropriate next step.

Answer E: Although beta-blocker titration may be part of long-term GDMT optimization, this patient's heart rate is already low-normal at 62 bpm, making further uptitration less desirable and potentially poorly tolerated.

Answer A/B: Initiation of additional guideline-directed medical therapies may be reasonable over time but would not address the immediate issue of LVAD-associated hypertension.

Answer D: There is no evidence of bleeding or another indication to discontinue aspirin.

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